Draft — pending clinical review

Using a spacer or a nebulizer at home

Get the medicine into the lungs — and know when to get help

The 5 things to remember

- An inhaler with a **spacer** works as well as a nebulizer for most attacks, and it is quicker.
- **Shake** the inhaler before **every** puff. Put in **one puff at a time**.
- Use a **mask** until about 4–5 years old, then a **mouthpiece**.
- In a nebulizer, use only the medicines and amounts your doctor wrote, and only **saline from the pharmacy**.
- If the reliever is needed **more often than every 4 hours**, see a doctor today.

How do I use the spacer?

1. Take off the cap and **shake** the inhaler well.
2. Push the inhaler into the back of the spacer.
3. Place the mask over the nose and mouth with a good seal, or put the mouthpiece between the lips.
4. Press the inhaler **once**.
5. Let your child take **5–6 slow breaths** in and out (or one deep breath and hold for 10 seconds if older).
6. Wait 30–60 seconds, shake again, and repeat for each puff the doctor prescribed.

How do I use the nebulizer?

1. Wash your hands. Sit your child upright.
2. Twist open the plastic vial (nebule) and squeeze all of it into the cup.
3. If your doctor told you to, add saline up to the amount written on your plan.
4. Fit the mask or mouthpiece and switch the machine on.
5. Your child breathes normally until the mist stops and the machine sputters, about **5–10 minutes**.
6. After a steroid nebule (budesonide), wipe your child's face and rinse the mouth with water.

How do I keep them clean?

- **Spacer:** once a week, wash in warm soapy water, do not rinse, and let it dry in the air. Rubbing it with a towel makes the medicine stick to the sides.
- **Nebulizer cup and mask:** after each use, rinse in warm water, shake off, and air-dry. Wash with soap once a day.
- Each child should have their own spacer, mask and nebulizer cup.

Common triggers in Saudi homes

- **Incense (bakhoor, oud) and tobacco or shisha smoke**. Keep them away from your child, including in the car.
- **Dust storms**: keep windows closed and your child indoors.
- Dust, cockroaches and pets in the bedroom; strong perfumes and cleaning sprays.

Get help now — call 997 or go to the emergency department if your child:

- Is too breathless to talk, feed or play.
- Has blue or grey lips.
- Has the skin between or under the ribs sucking in hard.
- Is very sleepy or confused.
- Does not improve after the reliever, or needs it again within 1–2 hours.

While you wait for help, keep giving the reliever as your child's asthma action plan says.

My child's plan

Reliever, and how many puffs or which nebule: _____

Preventer, and how often: _____

Mask or mouthpiece · saline top-up (mL): _____

AA

Prepared and medically supervised by Dr. Albaraa Albawab, Family Medicine
Founder & Medical Editor, Family Medicine Guides

This leaflet gives general information. It does not replace advice from your own doctor, who knows your health. Bring it to your next visit and ask about anything that is unclear. Written with AI assistance, checked by the medical editor, and still under review with colleagues.

Last updated: November 2026 · **Next review:** November 2027, or sooner if guidance changes

Sources:

1. Global Initiative for Asthma (GINA). Global Strategy for Asthma Management and Prevention, 2026 update
2. Al-Moamary MS, et al. The Saudi Initiative for Asthma (SINA) — 2021 update. Ann Thorac Med 2021;16:4–56
3. Cates CJ, et al. Holding chambers (spacers) versus nebulisers for beta-agonist treatment of acute asthma. Cochrane Database Syst Rev 2013
4. Product information: salbutamol, ipratropium and budesonide nebules; spacer and nebulizer manufacturer instructions